

[SYNTHETIC SAMPLE] Fictional record generated for the DELPHi prototype demo. Names, institutions, and statements are invented; the layout imitates a generic field-medical format. Not a real document and not derived from any real record.

[CONFERENCE REFERENCE] The meeting named above is a real, publicly announced scientific congress and is used here only as the setting of this fictional record. No session content, abstract, poster, or presented data from that meeting is reproduced or paraphrased; every person and conversation below is invented.

Congress Attendance Report — American Neurological Association Annual Meeting (ANA) 2025

Meeting period: September 2025 | Author: Dana Cho

Scope: unsolicited conversations held on-site. Session content and presented data are deliberately not reproduced here.

Conversation — Bram Merriwether, MD (Willowmere Neuroscience Center)

Stopped to talk on the way out of a session block, with no agenda on either side. Took a moment to say that one adult who had exhausted the usual combinations is now doing markedly better. Referral patterns into this clinic have changed, and newly referred patients is a bigger share than it was.

Conversation — Yannick Harkness, MD (Granite Bay Epilepsy Center)

Introduced by a colleague from the same region and talked for a few minutes. Mentioned a case series someone forwarded but did not raise any specific finding. Asked whether I had seen a guideline update discussed at journal club; I had not, and offered nothing in return. Noted an adult patient describing persistent dizziness after the last titration step.

Conversation — Orla Steinmetz, MD (Sable Creek Medical Group)

A hallway conversation during the afternoon break. Brought up an adult patient describing persistent dizziness after the last titration step. Asked whether I had seen a guideline update discussed at journal club; I had not, and offered nothing in return.

Conversation — Anouk Underhill, MD (Highmoor Health Neurology)

Caught between sessions in the poster hall; the conversation was short and entirely unprompted. Wants a concise interaction summary for elderly patients on several concomitant medications. Benefit verification is the step that most often delays a start here.

Conversation — Silvio Wycliffe, MD (Highmoor Health Neurology)

Approached me rather than the other way around, which is worth noting. Mostly small talk about the program and the venue. Offered, unprompted, that the longest-standing refractory patient in this panel has had the quietest stretch in years. Monitoring intervals during titration are set by clinic routine here rather than by protocol.

Conversation — Zubin Castellane, MD (Marbury Neurological Care Center)

Caught between sessions in the poster hall; the conversation was short and entirely unprompted. Closed by asking for something the clinic pharmacist could use when clearing older patients.

Referenced a review that circulated on teh service in passing.

Conversation — Quentin Kilbride, MD (Lakeshore Neurological Care Center)

Stopped to talk on the way out of a session block, with no agenda on either side. Referral patterns into this clinic have changed, and the refractory group is a bigger share than it was. Described a preference for slower titration in general, unrelated to any particular patient. Follow-up requested — a shorter interaction reference that intake staff could work from.

Conversation — Rohan Thistlewood, MD (Foxglove Health Neurology)

A hallway conversation during the afternoon break. Asked whether I had seen a session summary from a colleague; I had not, and offered nothing in return. Offered, unprompted, that several drug-resistant adults finally responded after years of cycling medications.

Conversation — Tadeo Ravenscroft, MD (Alder Point Neurology Associates)

Caught between sessions in teh poster hall; the conversation was short and entirely unprompted. Referenced a case series someone forwarded in passing. Asked whether I would be at teh meeting later this year. The patient portal is being piloted this quarter, and that took up teh first part of the conversation.

Conversation — Gemma Oakhurst, MD (Palmetto Neurology Institute)

A hallway conversation during the afternoon break. Raised a patient with a rash-like eruption that is being watched. Noted that the appeals process takes longer than the clinical decision and that this is not specific to any one product.

Conversation — Niamh Inverness, MD (Kestrel Comprehensive Epilepsy Clinic)

Stopped to talk on the way out of a session block, with no agenda on either side. Benefit verification is the step that most often delays a start here. Asked me to send caregiver-facing education material for older patients. Titration habits came up in general terms; no specifics were discussed and none were offered.

Conversation — Saoirse Wetherby, MD (Willowmere Neuroscience Center)

Introduced by a colleague from the same region and talked for a few minutes. Referenced a case series someone forwarded in passing. Referral patterns into this clinic have changed, and newly referred patients is a bigger share than it was.

Follow-ups have been logged in the interaction system.